

Total Rewards – Corporate Clinic – Guidelines and Procedures – Philippines**For RealPage Internal Use Only****Classification:** Internal Use**Document Control**

Field	Controlled Document Information
Title	Total Rewards – Corporate Clinic – Guidelines and Procedures – Philippines
Description	Guidelines and procedures governing the operation of the RealPage Philippines corporate clinic, including medical consultation, teleconsultation, fit-to-work evaluation, medical certificate validation, emergency room conduction, and accident reporting processes.
Document Type	Guidelines and Procedures
Owner	HR – Total Rewards
Audience	RealPage Philippines employees and company vendor-provided clinic staff
Country	Philippines
System of Record	Approved SharePoint document library associated with the designated Total Rewards team
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Approver	Anthony S. Balingit
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Reason for Change	Governance remediation to align document title, metadata, ownership, lifecycle, version-history, and system-of-record requirements with the Global HR Document Management Policy.

1. Purpose

Having a corporate clinic at RealPage Philippines, Inc. serves as a valuable resource for employees in the Philippines. By providing convenient access to healthcare services within the workplace, the company aims to promote employee health and well-being, leading to increased productivity and morale.

This document contains the corporate clinic processes and procedures intended to support preventive care services and early interventions, help maintain a safe and healthy workplace, and support adherence to applicable Occupational Safety and Health Standards requirements based on the OSH Law (DOLE-BWC).

This document serves as a guide to medical staff regarding the standard operating procedures applicable to Clinic Management services.

The contents of this document are not definitive and should be supplemented by applicable guidelines that may be issued by relevant government agencies from time to time.

2. Scope

The provisions of this document apply to:

- All employees of RealPage Philippines, Inc.
 - All clinic staff provided by the company's vendor partner, whether permanent or reliever.
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3. Definitions

Employee

Refers to all personnel of RealPage Philippines, Inc.

Local Operations Leaders

The employee's Supervisor, Manager, and Director based in the Philippines.

Occupational Health Nurse (OH Nurse)

A nurse duly licensed to practice their profession in the Philippines and possessing all additional qualifications required under Rule 1964.01 of the Occupational Safety and Health Standards.

Occupational Health Doctor (OH Doctor)

A physician duly licensed to practice their profession in the Philippines and possessing all additional qualifications required under Rule 1964.01 of the Occupational Safety and Health Standards.

Medical Certificate

A document issued by an attending physician that attests to the result of a medical examination of a patient. It may serve as documentation that an employee is unfit for work or as evidence of a health condition.

The certificate must be validated by the corporate clinic doctors where validation is required under the process described in this document.

Fit to Work Evaluation

An objective examination of an employee's health to determine whether the employee is fit to return to work.

A fit-to-work clearance is issued by the attending physician following the evaluation and must be submitted to and validated by the clinic doctors in accordance with the applicable company process.

Health Case Management (HCM) Tracker

A census of employees who visit or call the corporate clinic for medical consultation.

4. General Guidelines and Procedures**A. Medical Consultation**

1. The employee visits the corporate clinic.
2. The OH Nurse greets and receives the employee.
3. The OH Nurse conducts an initial assessment and properly records the information in the patient medical record.
4. The OH Nurse identifies whether the employee needs to be recorded in the Health Case Management (HCM) Tracker.
5. If the OH Doctor is on duty, the OH Nurse refers the employee to the OH Doctor for consultation.
6. If the OH Doctor is off duty, the OH Nurse refers the employee to another corporate clinic for consultation.
7. The OH Doctor provides an assessment, diagnosis, and appropriate medical advice and determines the final disposition.

Possible dispositions include:

- Return to workstation
- Medicine dispensing
- Clinic rest
- Send home
- Emergency Room (ER) conduction

Note: In the absence of the OH Doctor, the OH Nurse may provide management and intervention within the OH Nurse's scope of practice.

A.1 Return to Workstation

The employee returns to the workstation following the medical consultation when medically appropriate.

A.2 Medicine Dispensing

1. The OH Doctor prescribes the medicine.
2. The OH Nurse dispenses the appropriate medication or medication prescribed by the OH Doctor.
3. The employee records the transaction in the medicine dispensing log sheet.

A.3 Clinic Rest

1. The OH Doctor recommends clinic rest.
2. The OH Nurse obtains the employee's emergency contact details.
3. The OH Nurse allows the employee to rest in the clinic based on the OH Doctor's recommendation, but not to exceed one hour.
4. The OH Nurse sends an email notification to the employee's Local Operations Leaders.
5. If the employee's condition improves after clinic rest, the employee is asked to return to the workstation.
6. If the employee's condition does not improve, the OH Doctor provides the final disposition, such as Send Home or ER Conduction.
7. The OH Nurse informs the employee's Local Operations Leaders.

Note: Time spent by employees during clinic rest is considered unpaid if it exceeds the employee's meal breaks.

A.4 Send Home

1. The OH Doctor provides further advice and prescribes medication, if applicable.
2. The OH Nurse gives further instructions to the employee.
3. The OH Nurse informs the employee's Local Operations Leaders.
4. The employee signs the waiver/release form.
5. Upon arrival at the residence or hospital, the employee submits the signed waiver form by email to the Corporate Nurse.
6. The employee is sent home and is required to present a medical certificate upon return to work.
7. If the employee was seen by an OH Doctor, whether by telehealth or onsite, the OH Doctor may issue the medical certificate for a simple or non-complex case, subject to the attending physician's assessment.
8. If the case is complex and the employee is referred to a specialist, the specialist issues the medical certificate.

Note: If there is no OH Nurse or OH Doctor in the corporate clinic, an OH Nurse or OH Doctor in another corporate clinic may make the recommendation to the Local Operations Leader.

A.5 Emergency Room Conduction Process

Non-Life-Threatening Cases

1. The OH Nurse informs the employee's Local Operations Leaders.
2. The OH Nurse obtains the employee's emergency contact details.
3. The OH Nurse informs the Security team for assistance.
4. The employee is transported to the nearest medical facility through an appropriate private conduction method, such as a company/private vehicle or Transportation Network Vehicle Service (TNVS).
5. The OH Nurse accompanies the employee.
6. Security notifies the Local Operations Leaders regarding the conduction.
7. The OH Nurse leaves a notice at the clinic indicating **OUT FOR ER CONDUCTION**.
8. The OH Nurse may wait for the employee's relative before leaving the medical facility if another medical staff member is available to manage the corporate clinic.
9. The OH Nurse submits the ER conduction report to the Local Operations Leaders within the shift.
10. The OH Nurse follows up on the employee's health status within 24 hours following conduction.

Life-Threatening Cases

1. The OH Nurse informs the employee's Local Operations Leaders.
2. The OH Nurse obtains the employee's emergency contact details.
3. The OH Nurse endorses the employee to the nearest medical facility and contacts the accredited ambulance provider, private vehicle, or TNVS as appropriate.
4. The OH Nurse informs the Security team regarding the ambulance conduction.
5. The employee is transported by ambulance, private vehicle, or TNVS.
6. The OH Doctor accompanies the employee to the medical facility. If the OH Doctor is unavailable, the OH Nurse accompanies the employee.

7. The OH Doctor or OH Nurse waits for the employee's relative before leaving the medical facility.
8. The OH Nurse submits the ER conduction report to the Local Operations Leaders within the shift.
9. The OH Nurse follows up on the employee's health status within 24 hours following conduction.

Note: If the OH Nurse is the only medical staff member managing the corporate clinic at the time of conduction, the OH Nurse informs the onsite first aiders so they may assist with situations requiring immediate attention.

B. Teleconsultation Process

1. The employee calls the clinic.
2. The OH Nurse greets the employee and accommodates the call.
3. The OH Nurse conducts an initial assessment and properly records the information in the patient medical record.
4. The OH Nurse determines whether the employee needs to be recorded in the Health Case Management (HCM) Tracker.
5. If the OH Doctor is on duty, the OH Nurse refers the employee for consultation.
6. If the OH Doctor is off duty, the OH Nurse refers the employee to another corporate clinic for consultation.
7. The OH Doctor provides an assessment, diagnosis, and appropriate medical advice.
8. The OH Doctor sends email documentation to the OH Nurse.
9. The OH Nurse forwards the documentation to the employee.

Note: In the absence of an OH Doctor, the OH Nurse may provide management and intervention within the OH Nurse's scope of practice.

C. Fit to Work Evaluation

Fit to Work Evaluation is intended to help determine whether employees returning from illness or injury are fit to resume work.

1. The employee visits the corporate clinic.
2. The OH Nurse greets and receives the employee.
3. The OH Nurse conducts an initial assessment and properly records the information in the patient medical record.
4. The OH Nurse determines whether the employee should be recorded in the Health Case Management (HCM) Tracker.
5. If the OH Doctor is on duty, the OH Nurse refers the employee for consultation.
6. If the OH Doctor is off duty, the OH Nurse may provide management and intervention within the OH Nurse's scope of practice.
7. An employee determined to be fit to work is allowed to return to the workstation.
8. An employee determined not to be fit to work is sent home.
9. The OH Nurse informs the employee's Local Operations Leaders.

Note: In the absence of the OH Doctor, the OH Nurse may schedule a teleconsultation with an OH Doctor from another worksite. The teleconsulting doctor assesses and manages the patient accordingly.

Upon the employee's return from sick leave, the employee consults the onsite OH Doctor. Following the evaluation, the onsite OH Doctor issues the fit-to-work determination as applicable.

The teleconsulting and onsite OH Doctors coordinate internally regarding the employee's case.

If the employee was referred to a specialist, the specialist issues the fit-to-work certificate.

Important: If a specific return-to-work date is not indicated on the initial medical certificate, the employee should have a follow-up consultation with the attending physician, whether an external physician or Corporate Doctor depending on the case, and obtain a separate fit-to-work clearance.

D. Medical Certificate Validation

Medical Certificate Validation is intended to support the authenticity and accuracy of employee health documentation and maintain a healthy, safe, and efficient workplace.

Not all medical certificates require validation. Only certificates issued by non-HMO-accredited doctors are subject to the validation process described below.

1. The employee submits a medical certificate to the clinic for validation.
2. The OH Nurse acknowledges receipt of the medical certificate within 24 hours.
3. The OH Doctor, or OH Nurse under the supervision of the OH Doctor, validates the medical certificate.
4. Minimum turnaround time is within the shift, while maximum turnaround time is one to three days.
5. In addition to validating the medical certificate, the OH Doctor reviews the attending physician's recommendations in the context of RealPage's business operations and recommends the most appropriate course of action.
6. The OH Nurse sends the results of the medical certificate validation to the employee's Local Operations Leaders.
7. The validation result includes the details of the person from the doctor's clinic who confirmed or denied the medical certificate.
8. The OH Nurse reviews the medical certificate for validity or erroneous details.

Note: An employee may return to work and remain productive while validation of the medical certificate is ongoing, provided the case is non-communicable or non-contagious.

RealPage reserves the right to request a copy of laboratory tests and medications.

Important: Medical certificates from doctors who do not have a physical clinic and are not part of a teleconsultation platform must include a copy of BIR 2303 and a sample blank official receipt indicating that the doctor's home address is their business address.

E. Accident Reporting Process

1. The employee visits the clinic for consultation and reports the accident or injury.
2. The OH Nurse greets and receives the employee.
3. The OH Nurse completes the Incident Report Form.
4. The OH Nurse reports the accident or injury to Security, OSH, HRBP, OSH Chair, and Site Leader.

5. HRBP submits the required report to DOLE.

Note: The Medical Consultation Process applies to clinic visits related to accidents requiring first-aid treatment.

Accident/Incident Reporting should occur within 24–48 hours.

5. Templates

The following supporting materials are associated with this controlled procedure:

A. Email Template to Employee's Local Operations Leaders

The approved email template should be maintained with this procedure or linked from its authoritative SharePoint location.

B. Waiver/Release Form

Use the approved Waiver/Release Form available from the designated authoritative repository.

C. Incident Report Form

Use the approved Incident Report Form maintained in the designated authoritative repository.

Governance Requirement for Templates:

Blank reusable templates are controlled materials and should be maintained from an approved authoritative source. Once completed with information about an individual employee, the resulting record must be managed as an Employee-Specific Record and must not overwrite the blank master template.

6. Records and Document Handling

The operational processes described in this document may generate employee-specific records, including but not limited to:

- Patient medical records
- Medical certificates
- Fit-to-work clearances
- Completed waiver/release forms

- Completed incident reports
- Health Case Management records
- ER conduction reports
- Other employee-specific clinic documentation

Completed employee-specific records must be retained as complete point-in-time records and must not be overwritten or replaced.

Where applicable, each employee-specific record should include or be associated with:

- Document title
- Employee identifier
- Effective date and/or execution date
- Employing legal entity
- Country/jurisdiction
- Document category
- Source/originating process or system
- Retention category, where technically feasible

Access to employee-specific records must be limited according to role and legitimate business need.

The authoritative storage location for each specific clinic record category must follow the HR document category-to-system mapping maintained by HR Operations / HR Compliance. Where the approved system of record for a clinic record is unclear, submit a People Assist request under **Document Governance – System of Record Question** before establishing a new repository.

7. Document Lifecycle and Governance

The designated Document Owner is responsible for:

- Maintaining the accuracy of this document.
- Ensuring required reviews occur.
- Coordinating required approvals.

- Maintaining the controlled metadata.
- Maintaining the document's revision history.
- Ensuring obsolete versions are retired appropriately.
- Ensuring only the approved authoritative version is used operationally.

This document must not be published or used as a newly revised operational version until the required owner and reviewer approvals are completed.

The approved SharePoint version is the single authoritative copy. Local desktop copies, email attachments, chat copies, and temporary collaboration versions are working copies only.

8. Document History

Version	Date	Change	Owner / Approver
1.0	09/01/2024	Draft prepared by Human Resources	Total Rewards
2.0	10/01/2024	Updated definitions for Medical Certificate and Fit to Work Evaluation. Updated procedures for Send Home, ER Conduction – Non-Life-Threatening, ER Conduction – Life-Threatening, Fit to Work Evaluation, Medical Certificate Validation, and Clinic Rest.	Total Rewards
3.0	11/01/2024	Historical change information was not included in the source document reviewed.	Total Rewards
4.0	11/01/2025	Current source version. Detailed reason for change and approval evidence were not included in the source document reviewed.	Total Rewards

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